



Clinical Emergencies at Queen Square (NHNN and RLHIM) - Patient Pathway

PPE

TO BE WORN
BEFORE ANY
CONTACT WITH
THE INDIVIDUAL

To be used in conjunction with UCLH Trust policy
Management of Cardiac Arrest for Adults and Paediatrics

Establish status of collapsed / unwell person -
Inpatient, Outpatient, Visitor or Staff

All require 2222 call & should be managed initially by cardiac arrest team

INPATIENT

Outside ward area

Including patient who has just been discharged but who has not yet left NHNN or RLHIM premises



2222

ABCDE assessment
MONITOR and **STABILISE**

Treat presenting symptoms according to NEWS and Adverse Clinical Signs

Arrest Team decide **level of care** required for further management
Ward
Recovery
Intensive Care

Liaise with

Clinical Site Practitioner
Responsible Medical Team
Nurse-in-charge of proposed clinical area

Patient presenting **directly** to NHNN following recent discharge with a **neurological problem** related to recent inpatient treatment (e.g. surgical wound breakdown, CSF leak) must be assessed by the Clinical Site Practitioner (Bleep 8240) + relevant 'on call'

Neurosurgery SpR
or **Neurology SpR**

A decision will then be made regarding medical management +/- admission to NHNN



CONTACT BLEEP NUMBERS

Clinical Site Practitioner 8240
Outreach Team 8277

OUTPATIENT



2222

ABCDE assessment
MONITOR and **STABILISE**

Consider
IV access
ECG
Bloods
ABG

Treat presenting symptoms according to NEWS and Adverse Clinical Signs

An **outpatient** may be admitted to NHNN if appropriate but this must be discussed with responsible Primary Team and the proposed clinical area
If decision to admit to NHNN - liaise with **Clinical Site Practitioner**

If **not** to be admitted to NHNN then patient transferred to UCH A&E via **London Ambulance Service (LAS)**

CLINICAL EMERGENCIES IN MOBILE MRI SCANNER



2222

Mobile scanner staff call 2222 from internal telephone line in mobile scanner stating:

'Cardiac Arrest, Queen Square, Eye Clinic corridor, ground floor Queen Mary Wing by dark blue lift'

Patient is removed from scanner using MR safe patient trolley and evacuated immediately into **Eye Clinic** corridor via blue wheel chair entrance doors. Scanner staff have two access cards to allow them access through the necessary doors

Standard patient trolley permanently stationed in Eye Clinic with oxygen suction and patient transfer slide
Any additional equipment and drugs required brought by the Arrest Team

Once stable, patient transferred to Recovery for ongoing care while awaiting transfer to UCH A&E

VISITOR or STAFF



2222

ABCDE assessment
MONITOR and **STABILISE**

Consider
IV access
ECG
Bloods
ABG

Treat presenting symptoms according to NEWS and Adverse Clinical Signs

A **visitor** or **staff** member should be transferred to UCH A&E via **London Ambulance Service (LAS)** as soon as safe and practical to do so

Liaise with Clinical Site Practitioner and Floor co-ordinator (Anaesthetic Consultant) to identify suitable clinical area for patient while awaiting arrival of LAS if necessary

If patient with known epilepsy returns to baseline after 'usual' seizure, decision may be taken by medical staff that transfer to A&E not required

- **Arrest Team** responsible for patient management until handover to **LAS**
- **LAS** will take over care when they attend
- Termination of resuscitation and end-of-life decisions should not be made until patient is under care of paramedic crew or in UCH A&E
- Queen Square Emergency Team do not accompany to UCH A&E
- Transfer to UCH A&E via **LAS** as soon as safe and practical to do so